

# Health Insurance Claims Analysis

## Executive Summary

Period: 2024-01 to 2024-07  
Scope: multi-country underwriter portfolio

|              |                  |                     |            |
|--------------|------------------|---------------------|------------|
| Total Claims | Total Paid (USD) | Total Premium (USD) | Loss Ratio |
| 12           | \$9,501          | \$9,000             | 105.6%     |

### Executive Summary

The portfolio's health claims performance indicates a loss ratio of 105.57%, signaling a financial imbalance where claims paid exceed premiums collected. A significant concentration of claims activity is observed with Dr Sulaiman Al Habib, accounting for 59.61% of total paid claims. Essential Primary Hypertension emerges as the top diagnosis, suggesting a prevalence of chronic condition management within the insured population. These

### Key Findings

- The loss ratio of 105.57% indicates claims paid (\$9,501) exceeded premiums (\$9,000), reflecting a negative underwriting margin.
- Dr Sulaiman Al Habib accounts for 59.61% of total claims paid, suggesting pro concentration risk and potential overutilization.
- Essential Primary Hypertension is the top diagnosis, pointing to chronic disease and associated long-term cost implications.

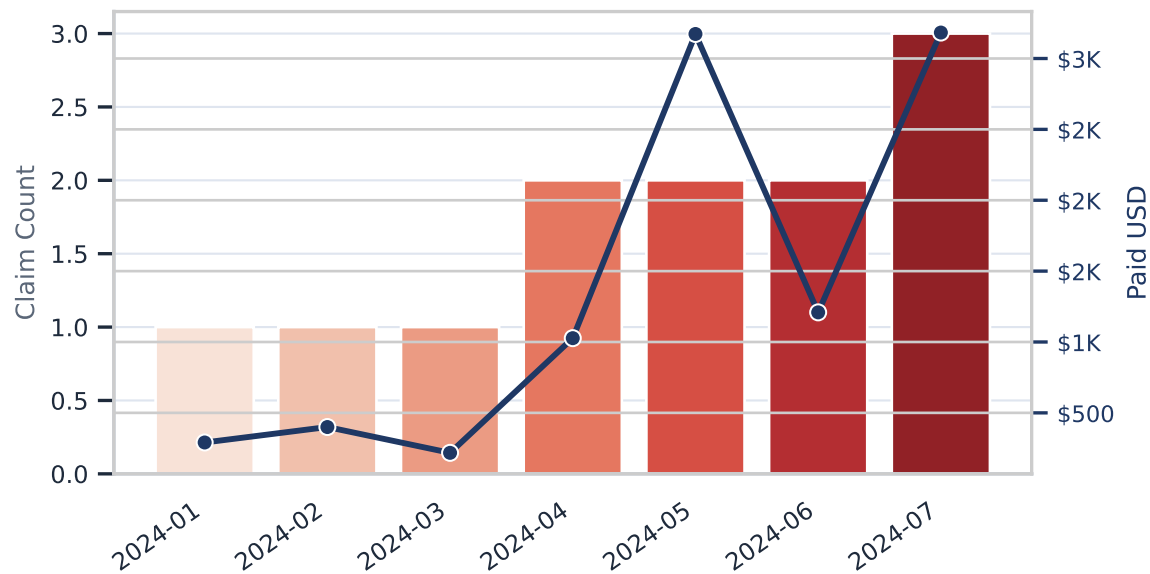
### Top Recommendations

- Engage with Dr Sulaiman Al Habib to review utilization patterns and negotiate cost-effective care pathways.
- Implement targeted wellness programs focused on hypertension management to reduce chronic disease progression and claims frequency.
- Conduct a portfolio-wide provider dependency analysis to identify and mitigate risks associated with high-cost providers.

# Health Insurance Claims Analysis

Claims Overview

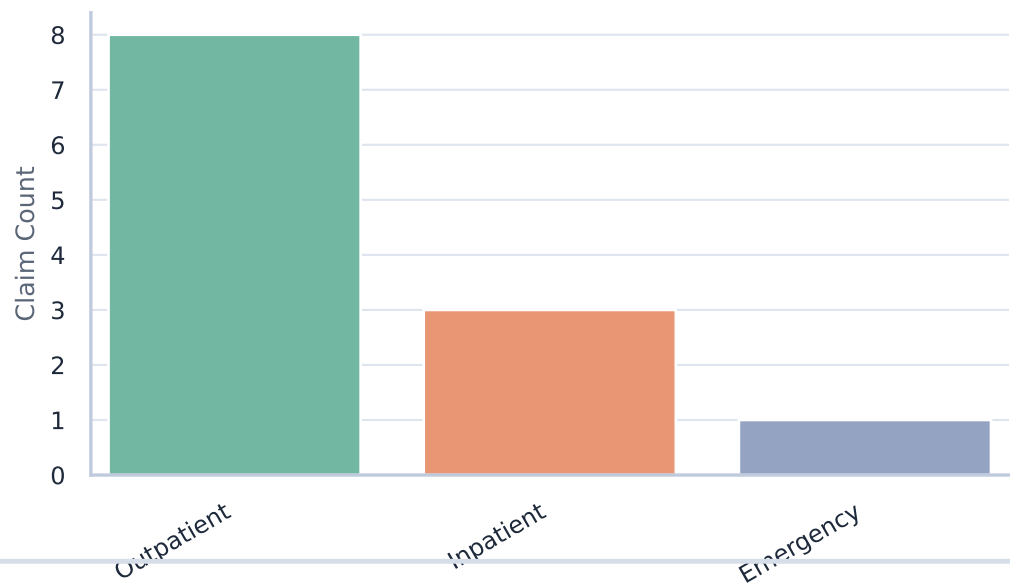
Monthly Claim Volume



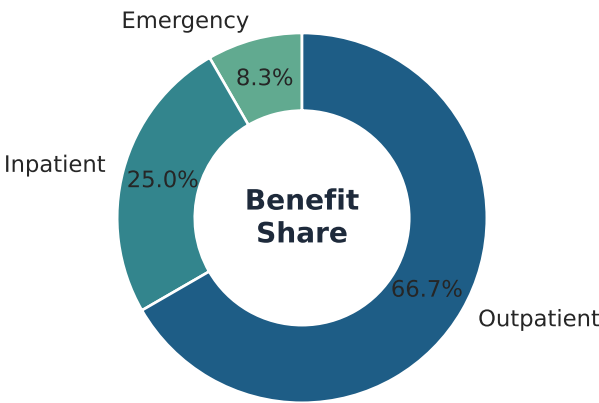
Quarterly Summary

| Quarter | Total Claims | Total Paid Usd | Avg Cost Per Claim |
|---------|--------------|----------------|--------------------|
| Q1      | 3            | 909            | 303                |
| Q2      | 6            | 5,409          | 902                |
| Q3      | 3            | 3,183          | 1,061              |

Benefit Mix by Claim Count



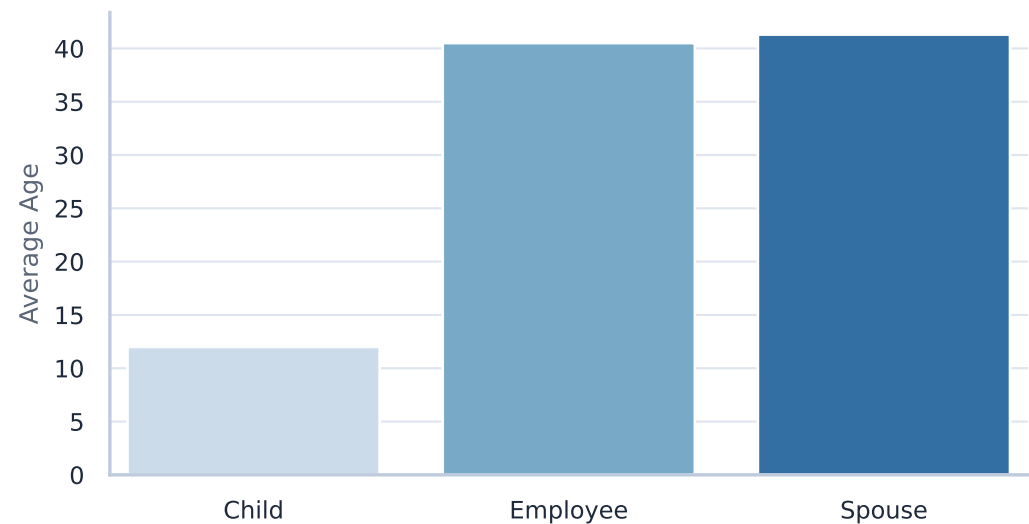
Benefit Distribution



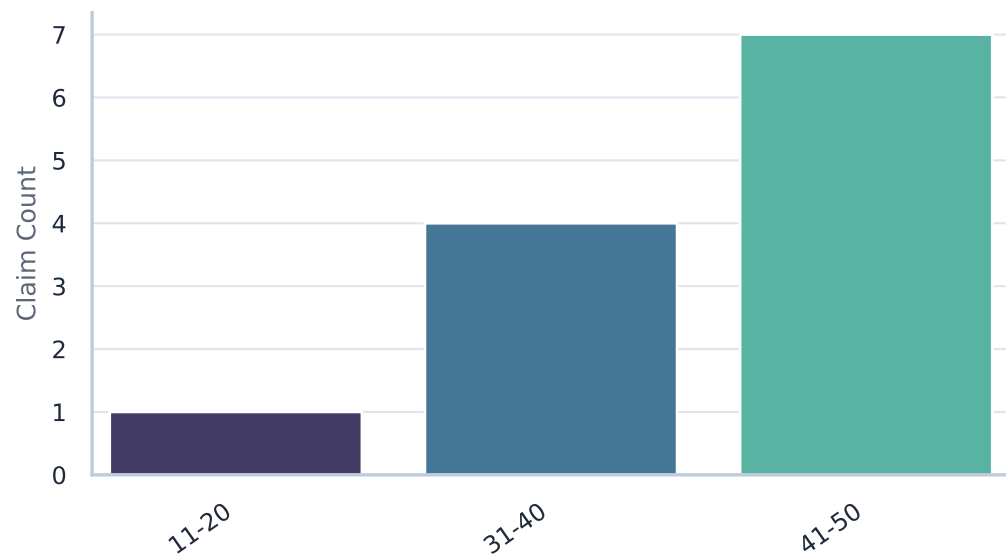
# Health Insurance Claims Analysis

Demographics and Cost Profile

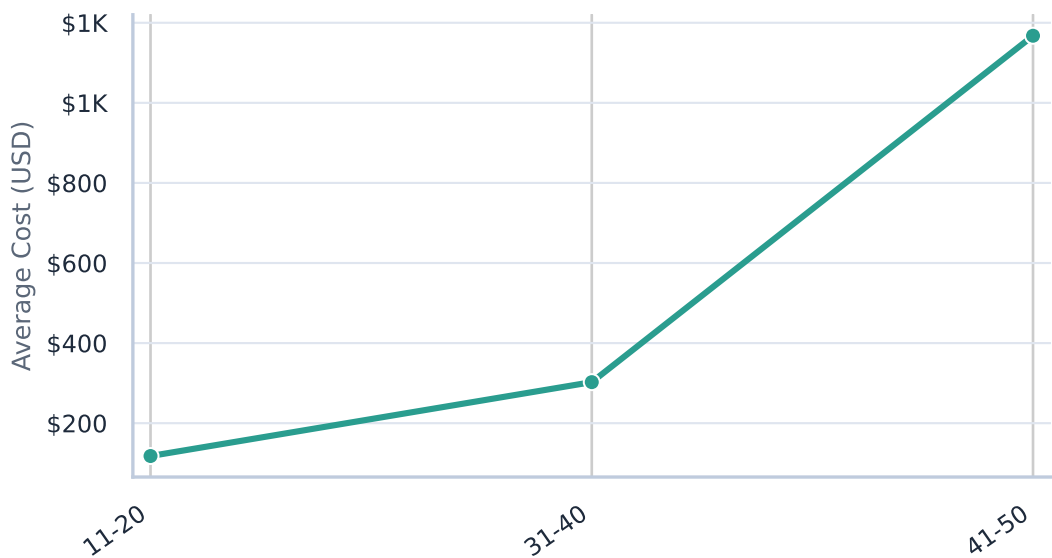
Average Age by Relationship



Claims by Age Group



Average Cost by Age Group



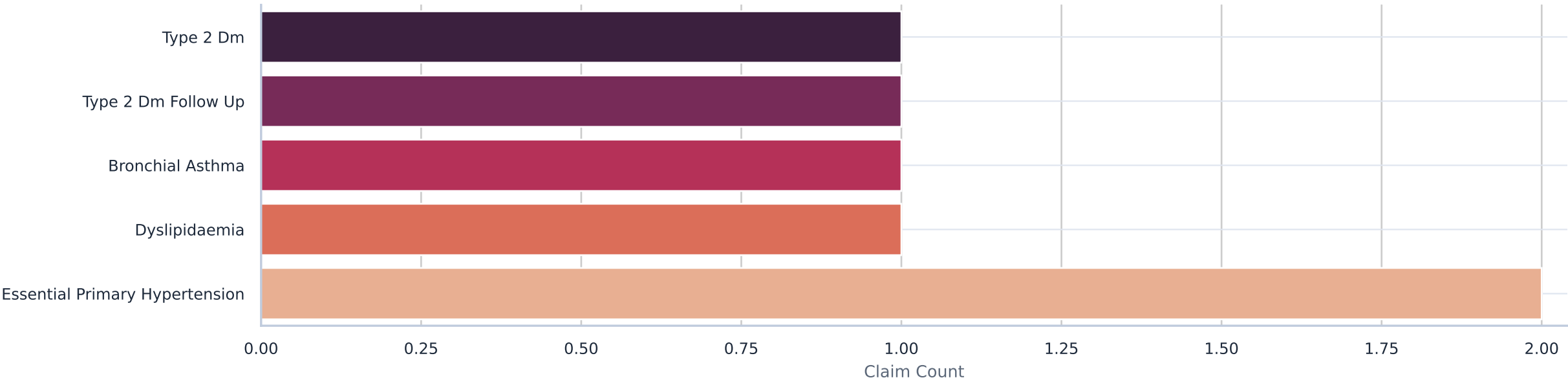
## Demographic Insights

- Highest claim-volume age group: 41-50.
- Highest average-cost age group: 41-50.
- Top provider concentration currently sits at 59.6%.
- Current portfolio loss ratio is 105.6%.

# Health Insurance Claims Analysis

PEC and Chronic Conditions

Top PEC Diagnoses by Claim Count



PEC Summary Table

| Icd10 Code | Diagnosis Description          | Total Claims | Total Paid Usd |
|------------|--------------------------------|--------------|----------------|
| I10        | Essential Primary Hypertension | 2            | 555            |
| E11        | Type 2 Dm                      | 1            | 400            |
| E11        | Type 2 Dm Follow Up            | 1            | 364            |
| J45        | Bronchial Asthma               | 1            | 300            |
| E78        | Dyslipidaemia                  | 1            | 218            |

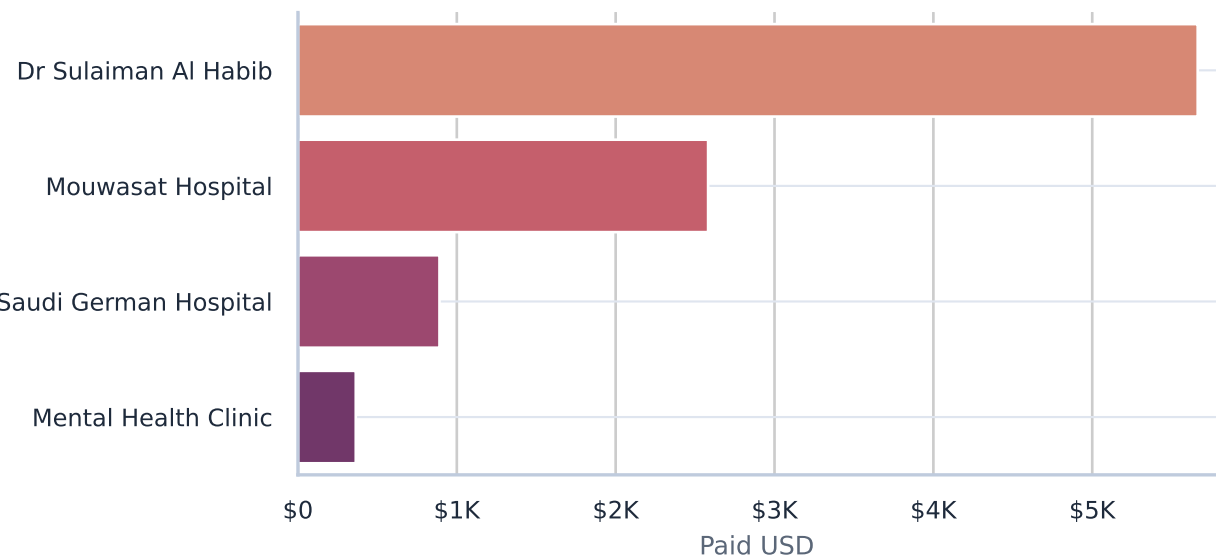
## Clinical Insights

- The high prevalence of Essential Primary Hypertension underscores the need for proactive chronic disease management strategies to reduce claims costs.
- Provider concentration with Dr Sulaiman Al Habib may indicate potential inefficiencies or overutilization, requiring closer monitoring and intervention.
- Claims data suggests a need for enhanced preventive care measures to address underlying health risks and

# Health Insurance Claims Analysis

Provider and Country Performance

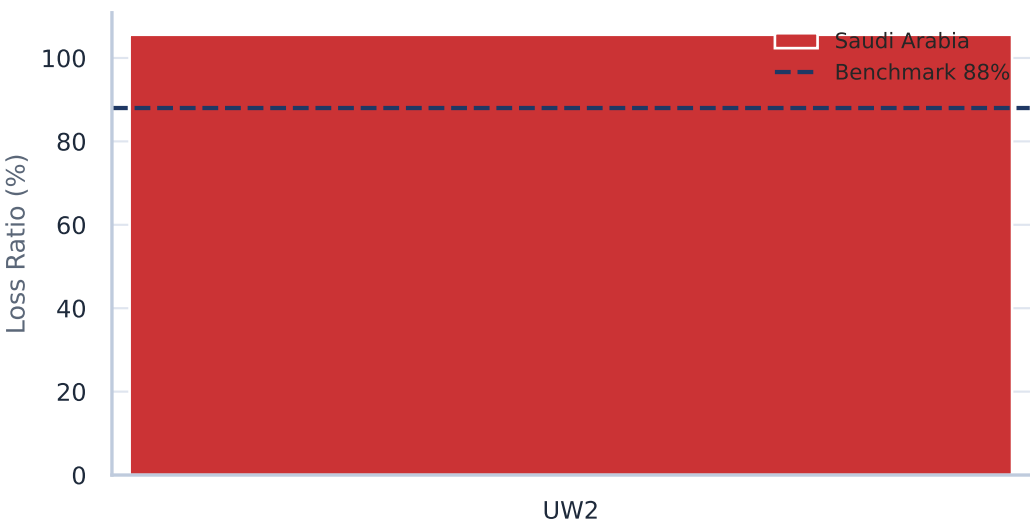
Top Providers by Paid Amount



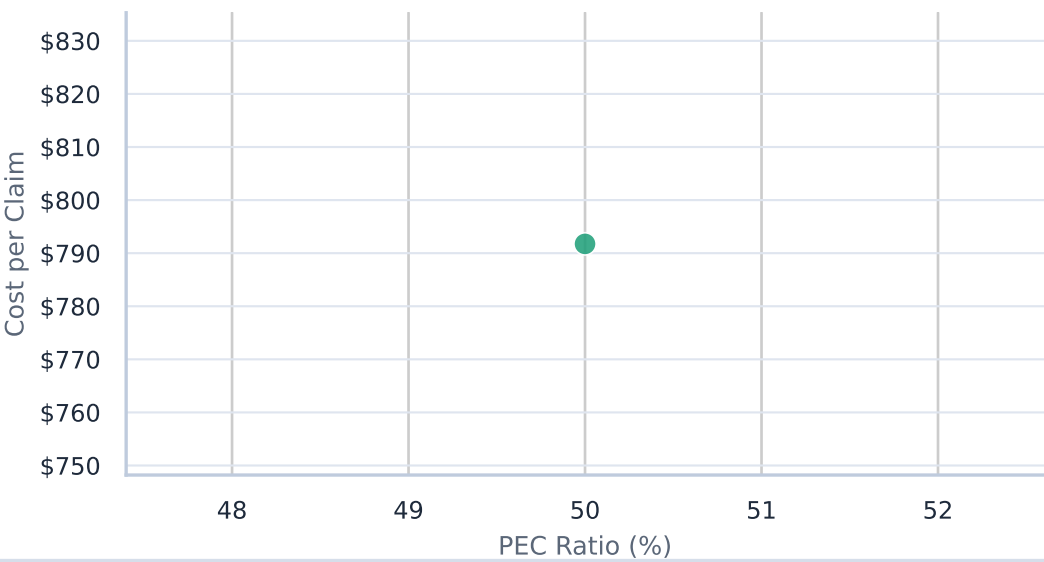
Provider Summary

| Provider Name         | Total Claims | Total Paid USD | Paid Share % | Avg Cost Per Claim |
|-----------------------|--------------|----------------|--------------|--------------------|
| Dr Sulaiman Al Habib  | 3            | 5,664          | 59.6%        | 1,888              |
| Mouwasat Hospital     | 4            | 2,582          | 27.2%        | 646                |
| Saudi German Hospital | 4            | 891            | 9.4%         | 223                |
| Mental Health Clinic  | 1            | 364            | 3.8%         | 364                |

Underwriter Loss Ratio Comparison



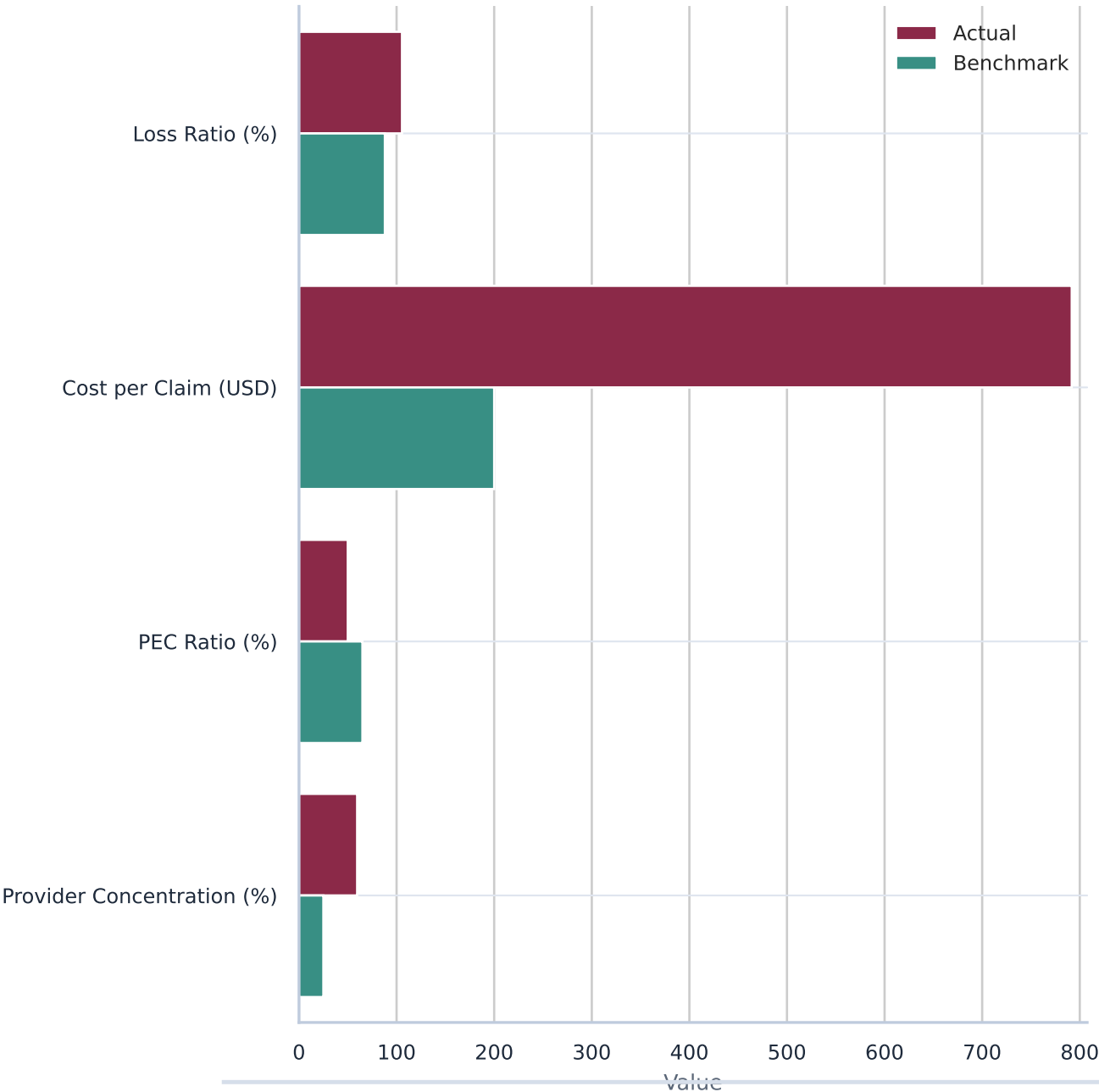
PEC Burden vs Cost per Claim



# Health Insurance Claims Analysis

Benchmark Positioning and Handoff

Actual vs Benchmark



## Handoff Notes

- PDF pack includes 8 pages with KPI, trend, demographic, and PEC analyses.
- Power BI-ready tables are generated in CSV format and can be connected directly.
- Narrative sections can run in fallback mode or use Azure OpenAI for richer wording.
- Use this report layout as a deterministic baseline for future underwriter uploads.

Action plan focused on chronic utilization, provider leverage, and country-specific controls.

### 1. Chronic Disease Management Programme

#### Business case:

PEC/Chronic conditions currently drive 50.0% of portfolio claims. Hypertension and diabetes contribute 2 diagnosed episodes.

#### Action:

Run annual HbA1c, blood pressure, and lipid screening with a regional wellness partner; use telemedicine follow-up for confirmed chronic members.

#### Est. saving:

USD 147 to 220 per year from a 10% episode reduction.

### 2. Provider Network Renegotiation

#### Business case:

Top provider concentration is 59.6% of paid amount, creating renewal pricing risk and negotiation dependency.

#### Action:

Request itemized billing audits for top-spend providers and negotiate a portfolio-level volume discount across countries.

#### Est. saving:

USD 340 to 510 per year with a 6%-9% discount range.

### 3. Qatar Portfolio Pre-Authorization Tightening

#### Business case:

Qatar loss ratio is currently 0.0% and top two high-cost claims contribute 0.0% of Qatar paid value.

#### Action:

Mandate pre-authorization for inpatient admissions above USD 1,500 in Qatar and add second-opinion review for oncology treatment plans.

#### Est. saving:

Risk mitigation action to prevent loss-ratio drift above renewal tolerance bands, especially where case concentration is high.

### 4. Class C / Bronze Network Consolidation

#### Business case:

Class C/Bronze segment in Saudi Arabia averages USD 364 per claim and can be managed with tighter provider panels.

#### Action:

Review Bronze networks and consolidate to fewer preferred providers (e.g., Mental Health Clinic + Mouwasat Hospital) after HR and employee communication sign-off.

#### Est. saving:

USD 87 to 131 per year via consolidation and admin efficiency.

Full KPI Summary

| Metric               | Value     | Benchmark      | Status       |
|----------------------|-----------|----------------|--------------|
| Total Claims         | 12        | -              | -            |
| Total Paid (USD)     | USD 9,501 | -              | -            |
| Loss Ratio           | 105.6%    | 85-95%         | Above range  |
| Cost Per Claim (USD) | USD 792   | USD 150-250    | Above range  |
| PEC/Chronic Ratio    | 50.0%     | < 65%          | Within range |
| Inpatient %          | 25.0%     | 25-35%         | Within range |
| Avg Age (Employee)   | 40.5 yrs  | 30-42 yrs      | Within range |
| Oncology Cases       | 2 claims  | < 5 per period | Within range |
| Maternity Cases      | 1 claims  | -              | -            |

Methodology and Currency Conversion

- Loss Ratio = Total Paid Amount (USD) / Gross Written Premium (USD) x 100.
- UW1 (Oman): Amounts in Baisa/Fils are converted to OMR then to USD as per pipeline rules.
- UW2 (Saudi Arabia): Amounts are treated as USD where already normalized in source.
- UW3 (Qatar): Amounts in QAR are converted to USD using configured conversion factors.

Disclaimer

- Prepared from underwriter-provided claim records for the selected reporting period.
- Benchmarks are directional and should be validated during renewal discussions.
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